A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

18 May 2026

Dr Lachlan McLean
Bunniyong Medical Centre

Dear Dr McLean,

Re: Mr Adam Voros
DOB: 08/10/2002 Phone: 0407 682 764
Address: 8 Ashley Court, ALFREDTON, VIC, 3350
Email: paula@kittelty.com.au

Summary

- **Chronic constipation and suspected pseudo-obstruction in the context of cerebral palsy and previous T4-L4 spinal fusion.**
- **For a trial of Rifaximin to address abdominal distension.**
- **For dietary interventions including green kiwifruit and hot caffeinated beverages to stimulate motility.**
- **For a trial of increasing stool softener dosage and adjusting the timing of Nexium to 30 minutes before breakfast.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Adam and his mother, Paula. Thank you for referring him.

Symptoms and history of presenting complaint

Adam presents with a complex history of bowel dysfunction, which has significantly worsened since his spinal fusion (T4 to L4) performed between in 2019. He has a history of suspected pseudo-obstruction and hospital admissions, though the current goal is to manage his symptoms proactively at home to avoid further acute presentations.

Currently, his bowel movements are frequent but often ineffective. On a typical day, he may have up to nine bowel movements, which are generally soft or liquid. However, he suffers from "impaction" events where a relatively hard stool "plug" forms. During these episodes, he repeatedly attempts to open his bowels without success, leading to significant distress and behavioural changes. To manage this, the family uses a combination of daily stool softeners and intermittent stimulant laxatives.

Adam experiences significant abdominal distension. While he does not frequently vomit and his weight remains stable, the bloating is a persistent feature. He also experiences faecal incontinence, which the family currently prefers over the risk of impaction and hospitalisation.

Diet

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
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Adam's diet includes apple and pear purees, which are high in FODMAPs and may contribute to his bloating, although they are used to assist with stool softening. He consumes one mocha in the morning (caffeine intake). His weight is stable, and his height is approximately 175-180 cm.

Past medical history and medications

Adam has a history of cerebral palsy, autism, and scoliosis requiring the aforementioned T4-L4 spinal fusion. He has epilepsy, which is generally well-controlled, though he recently had an unusual episode. He has also received a recent diagnosis of atrial fibrillation.

His current medications include:

- Epilim (sodium valproate)
- Risperidone (Risperdal)
- Nexium (esomeprazole) 40 mg (taken at night)
- Coloxyl (docusate) without Senna
- Magnesium oxide capsules
- Pico-Prep (sodium picosulfate) used intermittently (one sachet)

Treatments trialled before and investigations performed

Adam has previously trialled prucalopride (Resotrans), but this was discontinued due to headaches and lack of efficacy. He has also trialled rifaximin approximately two years ago, linaclotide, and various other laxatives with limited success. He has previously undergone a barium swallow and investigations by several specialists, including Prof Geoff Herbard.

Social history, family history

Adam lives at home with his mother, Paula and father, who provides significant care. There is a family history of stomach cancer in his grandfather (a smoker), but no known family history of inflammatory bowel disease or coeliac disease.

Examination

This was a telehealth consultation; therefore, a formal physical examination was not possible. However, Adam appeared stable on screen, and his mother provided a detailed account of his abdominal distension and bowel patterns.

Impression and plan

My impression is that Adam’s symptoms represent a neurogenic bowel and motility disorder, likely exacerbated by the mechanical stiffness and potential nerve involvement following his high-level spinal fusion. While there is a risk of sigmoid volvulus in patients with this degree of redundancy and pseudo-obstruction, we will focus on medical and dietary optimisation to avoid the need for surgical intervention or a stoma (uncommon but possible), which would be difficult to manage.

1. **Rifaximin:** I have prescribed a two-week course of Rifaximin to see if this reduces his abdominal distension. This may require sourcing from a compounding pharmacy.

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2. **Dietary Changes:** We discussed introducing two green kiwifruits daily (without skin), as these have a better evidence base for motility than apple or pear purees and are less irritating. I have also suggested adding a second hot mocha or a hot decaffeinated drink in the morning to stimulate the gastrocolic reflex.
3. **Medication Adjustments:** We will trial increasing his stool softener (either Coloxyl or Magnesium Oxide) by one tablet daily to see if we can reduce the frequency of Pico-Prep use.
4. **Nexium:** I have advised moving his Nexium dose to 30 minutes before breakfast to maximise its efficacy, as it is currently approximately 50% less effective when taken after food or at night.
5. **Follow-up:** I will review Adam in approximately four to six months to assess the response to these interventions.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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